جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : غاده رسمي محمد صقر
المحافظة : كفر الشيخ **الحي / المدينة :** دسوق **العنوان :** دسوق
رقم البطاقة : 27503011503083 **رقم التليفون :** 01114789635 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 01-03-1975 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: غاده رسمي محمد صقر
National ID: 27503011503083

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

PATHO Egypt
باثو إيجيبت
لتحاليل الأنسجة والأورام

أ.د. نجلاء سمير أ.د. نهال رضوان أ.د. فاطمة سامي
أساتذة الباثولوجي - كليه الطب-جامعه عين شمس

Patient Name	غاده رسمى محمد صقر	Lab.No.	26H009
Age / Gender	Female - 51 Y	Registered Date	2026-08-03 08:48 PM
Referred By	أ.د. محمد الغريب	Reporting Date	2026-08-08 05:00 PM

Histopathology Report

Specimen

Right breast mass (U/S guided tru cut biopsy).

Gross

Received is a grayish yellow tissue core measuring 1.2 cm long, totally submitted as such in one cassette.

Microscopic

Sections examined revealed fibrofatty breast tissue core infiltrated by an invasive carcinoma made up of sheets and clusters of malignant epithelial cells with focal tubular differentiation. The cells show moderate pleomorphism and active frequent mitosis. No in-situ ductal carcinoma (DCIS) observed. The tumor is set in a desmoplastic inflamed stroma. No definite lympho-vascular invasion detected. No perineural invasion identified in the examined sections.

Diagnosis

Right breast mass ; U/S guided tru cut biopsy:
Invasive duct carcinoma (NST),Grade II.

Immunohistochemical Profile:
- Estrogen Receptors (ER): Positive strong nuclear staining in 90 % of tumor cells, (Allred score 8/8).
- Progesterone Receptors (PR): Positive strong nuclear staining in 70 % of tumor cells, (Allred score 8/8).
- HER2-neu: Positive (Score 3+): Comlplete ,intense circumferential membranous staining observed in > 10% of tumor cells.
- Ki-67: Proliferation index is 30% of tumor cells.

Dr. Nehal Radwan
Dr. Nehal Radwan

This report is electronically signed

pathoegypt@gmail.com
www.pathoegypt.com
٤٠ ش الحجاز ـ مصر الجديدة - الدور الأول ـ شقة ١١٠
٠١١٤٤٤٢٨٩٣٣ / ٠٢٢٢٤٠٦٧٩٩

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

- *Left adnexal functionary cyst with FDG activity (4.1x4.3cm) with max. SUV~ 22.1) with enlarged uterus for clinic-laboratory assessment.*

- *Physiologic FDG excretion is seen in the kidneys & bladder.*

Musculoskeletal:

- *Metabolically active FDG avid osseous wide spread, diffuse, the whole axial, appendicular skeleton) ,mixed lytic & sclerotic lesions) the most active is seen left proximal humeral shaft (with max. SUV~ 37.2) for follow up.*

Conclusion:

- ***Positive PET/CT study shows:***
- ***Metabolically active FDG avid right breast inner quadrant speculated soft tissue neoplastic mass with low active FDG avid right axillary L.Ns for histopathology & follow up.***
- ***Metabolically active FDG avid nodal, hepatic, osseous lesions (likely metastatic) , for follow up.***

Thank you for your reference.

PET/CT Unit

Radiology consultant

Dr , Sherien Mahdy , M.D

Nuclear Medicine consultant

Dr/Ahmed Touhamy, M.D

Revised by
Dr/ Mohamed Emam

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: غاده رسمى محمد صقر *Date:15/04/2026*
Referring Physician: Prof. Dr.

Whole body PET/CT

Reason of study:
Assessment response of the therapy
Radiopharmaceutical:
F-18-FDG 10 mCi injected I.V.
Fasting blood glucose:
112 mg/dl
Technique:-
Approximately one hour following the IV injection of the radiopharmaceutical, a total body PET – CT was acquired from mid thigh to base of skull with the arms extended above the head. CT was acquired for the purpose of anatomic localization & attenuation correction. Data was processed & displayed according to departmental protocol.

REPORT:

Hepatic Reference: max SUV ~ 4.1.
Head & Neck:
Still seen:

- *The brain exhibited normal FDG bio-distribution.*
- *Physiologic FDG uptake is seen in the oropharynx, salivary glands & larynx.*
- *No focal diffuse thyroid gland FDG uptake could be elicited.*
- *No metabolically active FDG avid cervical LNs*

Chest:

- *Metabolically active FDG avid prevascular, right para tracheal, precaval, scattered mediastinal L.Ns (the prevascular L.N (1.4x1.9cm) with max. SUV~ 7.5).*
- *Bilateral lungs atelectatic changes, consolidation opacities (FDG activity) with max. SUV~ 7.4) with noted bilateral basal mild metabolically inactive pleural effusion (inflammatory).*

PET Scan
[[DOCTYPE:PET_SCAN]]

- *No metabolically active FDG avid mediastinal or hilar lymph nodes are seen.*
- *There are no pleural or pericardial abnormalities.*
- *Physiologic FDG uptake is noted in the myocardium.*

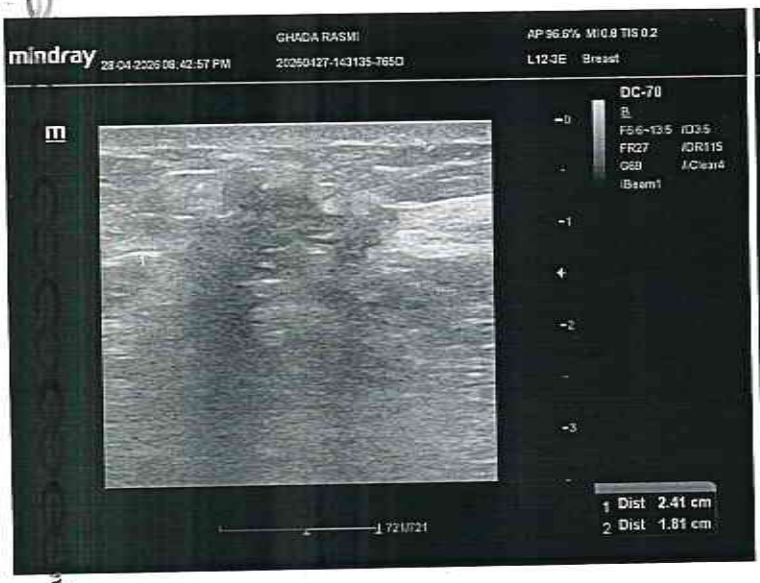
Breast & axilla:

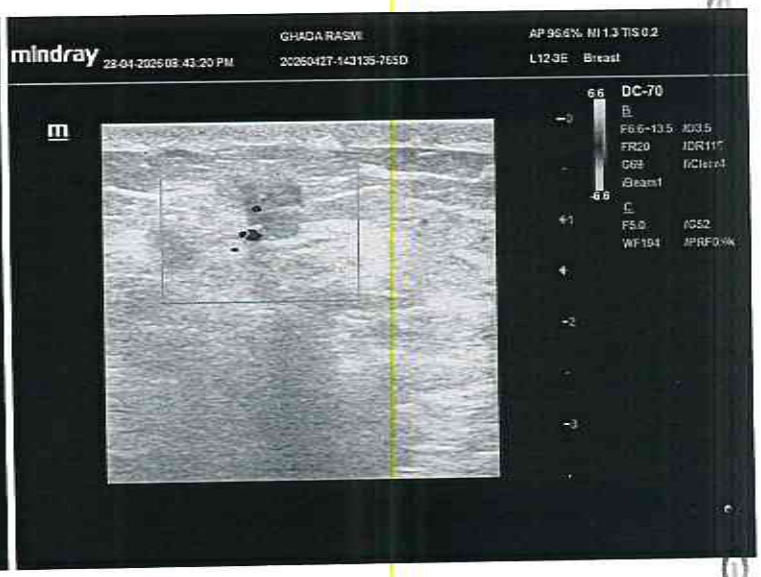
- *Metabolically active FDG avid right breast low inner quadrant speculated neoplastic soft tissue mass, measuring (2.1X2.7cm) with max. SUV~ 15.4) with minimal skin thickening, no FDG activity for histopathology.*
- *Few right sided axillary low active FDG avid L.Ns (up to (1.5cm) with max. SUV~ 3.4.*
- *Clear appearance of the left breast.*
- *Normal finding of left axilla.*

Abd & pelvis ;

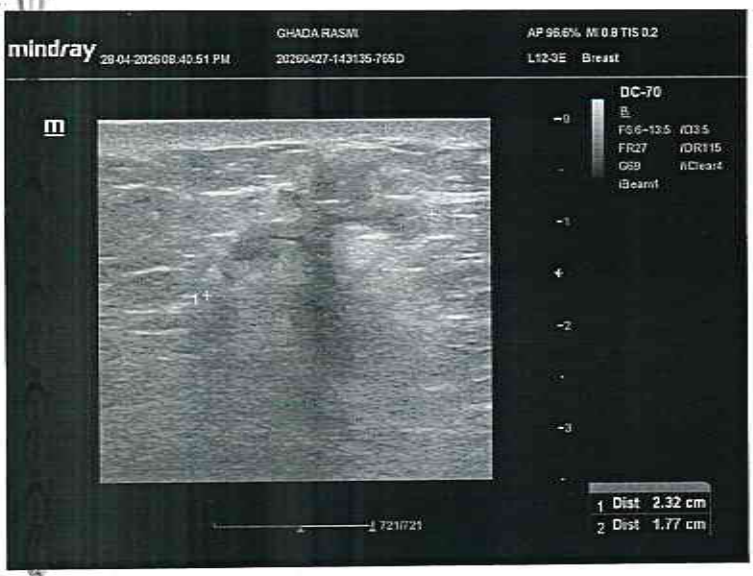
- *Hepatomegaly with noted multiple metabolically active FDG avid hepatic bilobar focal hypodense lesions , (the largest & most prominent is seen at segment VII , measuring (2.5x2.1cm) with max. SUV~ 19.4) for follow up.*
- *Cholecystectomy.*
- *Multiple low grade FDG avid aorto-caval, bilateral para aortic left common iliac subcentimetric low grade FDG avid lymphadenopathies (with max. SUV~ 4.2).*
-

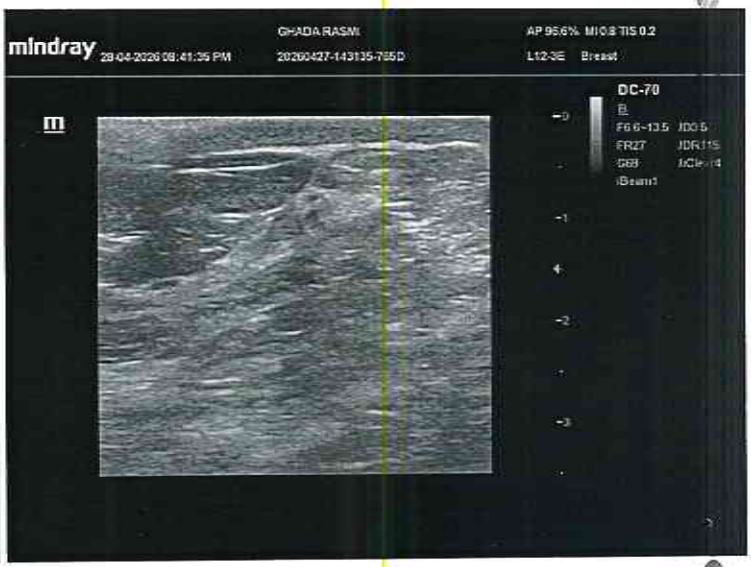
PET Scan
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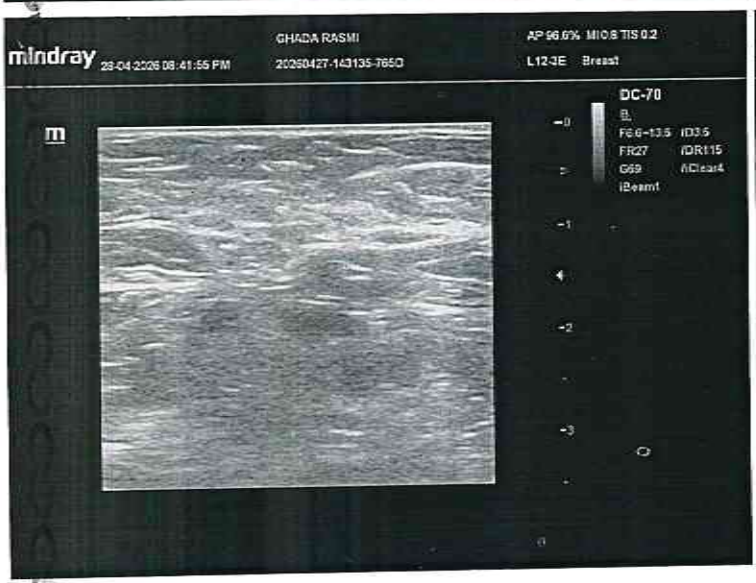


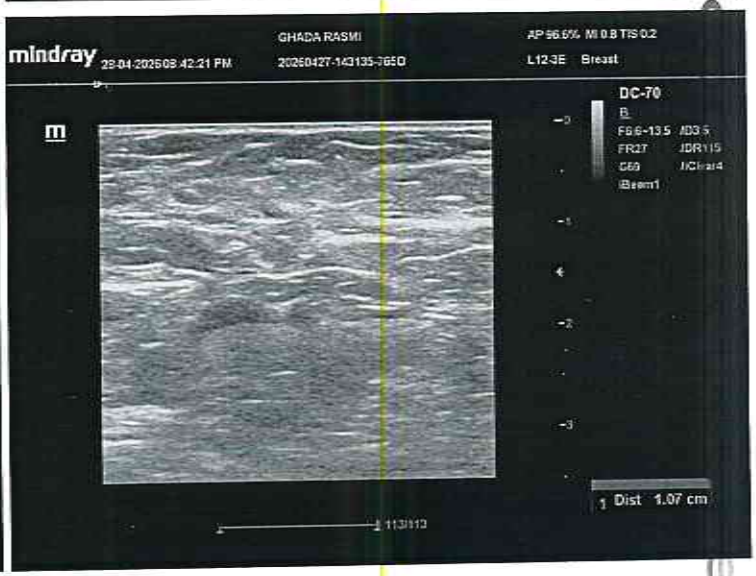


Ultrasound
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Ultrasound
[[DOCTYPE:ULTRASOUND]]

Name: GHADA RASMI MOHAMED	Date : 27 /4/2026
Age 50	Exam MAMMO

MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND FOR BOTH BREASTS REVEALED:

Clinical complaint : right breast lump

- The right breast is the seat of highly suspicious ill defined irregular mass , at the lower outer quadrant of the breast . No diffuse skin thickening , small associated suspicious micro calcification visible. Ultrasonography for the right breast showed an inhomogeneous poorly defined mass in the right lower outer at infra mammary fold measuring 20x25 mm in diameters with increased vascularity and surrounding area of mild architectural distortion .
- Small mildly enlarged right axillary lymph node the largest 11 mm
- Normal left mammogram.
- Normal skin thickness and contour of the left breast.

OPINION:-

Picture is in favor of a suspicious mass in the right breast ? multi focal , for dynamic MRI and biopsy BIRADS 4c
Biopsy can be done upon request

Best regards,
DR. Eman El Taweel .MD

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

BIRADS 0 : Needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2: Benign.
BIRADS 3: Probably benign.
BIRADS 4: A: mild Probability of malignancy
B: moderate Probability of malignancy
C: marked Probability of malignancy.
BIRADS 5: Malignant.
BIRADS 6: Proven pathologically malignant

Mammogram
[[DOCTYPE:MAMMOGRAM]]

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Radiology consultant

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Nuclear Medicine consultant

Dr/Ahmed Touhamy, M.D

Revised by
Dr/ Mohamed Emam

PET Scan
[[DOCTYPE:PET_SCAN]]

[[DOCTYPE:CT_SCAN]]
CT Scan

جمعية مسجد د. مصطفى محمود
Dr . Mostafa Mosque Society
الوحدة العلاجية
مركز الأشعة المقطعية والرنين المغناطيسي

Name: Ghada Rasmy Mohamed ***Date:- 29/1/2026***

R:

Prof DR.

FOLLOW UP NON CONTRAST C.T. SCAN OF THE CHEST

REVEALED :-

- *In comparison with last dated 18/12/2025 the current study show the following data:*
- *Stationary course regarding the previously noted wide spread patchy areas of air entrapment , peri bronchial cuffing , traction bronchiectosis , coarsed interstitium and few thin fibrotic bands.*
- *No pleural effusion.*
- *Still noted few enlarged hilar and mediastinal LNs.*
- *Average sized heart with newly developed pericardial collection.*
- *Post cholecystectomy.*
- *Suspected right adrenal mass lesion.*

CONCLUSION:-

- ***Stationary course since last study dated 18/12/2025 with newly developed mild pericardial collections .***

Dr. Fatma Naeem ***Prof .Dr. Tamer Ahmed Kamal***

E.S



National ID – Front
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National ID – Back
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